

1. Administrative & Study Identification

Full Study Title: A phase II, multicenter, randomized, double-blind study of tobemstomig/R07247669 combined with nab-paclitaxel compared with pembrolizumab combined with nab-paclitaxel in participants with previously untreated, PD-L1 positive, locally-advanced unresectable or metastatic triple-negative breast cancer

Short Title/Acronym: Study of Tobemstomig + Nab-Paclitaxel vs Pembrolizumab + Nab-Paclitaxel in TNBC **Protocol Number:** C044194 (2022-502457-34-00) **NCT Number:** NCT05852691 **Posting ID:** 725224621758113085 **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Sponsor/Company:** Roche (Hoffmann-La Roche) **Investigational Product:** Tobemstomig (R07247669) + Nab-paclitaxel **Phase of Development:** Phase II **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy (measured by Progression-Free Survival) and safety of a novel immunotherapy candidate, tobemstomig, in combination with nab-paclitaxel compared to pembrolizumab plus nab-paclitaxel in patients with previously untreated PD-L1-positive TNBC. **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DOR), and the pharmacokinetic/pharmacodynamic profile of tobemstomig.

2.2 Background & Rationale Triple-negative breast cancer (TNBC) is a type of cancer that does not respond to hormonal treatments or treatments targeting the HER2 protein. While current immunotherapies like pembrolizumab combined with chemotherapy can be effective, these treatments can stop working over time, highlighting a need for better treatment options. This study evaluates whether tobemstomig (an experimental drug), when used in combination with nab-paclitaxel chemotherapy, will provide better outcomes and prolong progression-free survival for patients with untreated PD-L1-positive locally advanced unresectable or metastatic TNBC compared to the current standard of care.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Pembrolizumab + Nab-paclitaxel) **Blinding:** Double-blind (neither participant nor clinical trial doctor knows the group assignment) **Randomization:** Randomized (1:1 Allocation)

3.2 Planned Enrollment & Duration Target **\$N\$:** Approximately 83 participants **Number of Sites:** Multicenter (Global, including Europe, US, and Rest of World) **Estimated Study Start:** March 2023 **Estimated Primary Completion:** 31-Oct-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Metastatic or locally advanced unresectable, histologically documented triple-negative breast cancer (absence of HER2-over-expression, ER, and PgR expression by local assessment); HER2-low-status. 2. Measurable disease per Response Evaluation Criteria in Solid Tumors (RECIST) v1.1. If metastatic disease (Stage IV), measurable disease outside of the bone. 3. No prior systemic therapy for metastatic or locally advanced unresectable TNBC. 4. Tumor PD-L1 expression as documented through central testing of a representative tumor tissue specimen. 5. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 6. Adequate hematologic and end-organ function. 7. Negative HIV test at screening (exception: individuals stable on anti-retroviral therapy, with CD4 count $\geq 200/\mu\text{L}$, and undetectable viral load are eligible). 8. Negative hepatitis B surface antigen (HBsAg) test at screening. 9. Positive hepatitis B surface antibody (HBsAb) test at screening, or a negative HBsAb accompanied by either a negative hepatitis B core antibody (HBcAb) or a positive HBcAb test followed by quantitative HBV DNA $< 500\text{ IU/mL}$. 10. Negative hepatitis C virus (HCV) antibody test at screening, or a positive HCV antibody test followed by a negative HCV RNA test at screening. 11. Adequate cardiovascular function.

4.2 Exclusion Criteria 1. Pregnancy or breastfeeding, or intention of becoming pregnant during the study or within 4 months after the final dose of toremifene/pembrolizumab, and 6 months after the final dose of nab-paclitaxel. 2. Poor venous access. 3. History of malignancy within 5 years prior to consent, except for the cancer under investigation and malignancies with a negligible risk of metastasis or death (e.g., adequately treated carcinoma in situ of the cervix, nonmelanoma skin carcinoma, localized prostate cancer, DCIS, or Stage I uterine cancer). 4. Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases or history of leptomeningeal disease. 5. Pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures (once monthly or more frequently). 6. Hypercalcemia or hypocalcemia that is symptomatic. 7. Active or history of autoimmune disease or immune deficiency. 8. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on screening chest CT scan. 9. Active tuberculosis (TB). 10. Significant cardiovascular/cerebrovascular disease within 3 months prior to consent, history or presence of a clinically significant abnormal ECG, or history of/risk factors for ventricular dysrhythmias. 11. Major surgical procedure within

4 weeks prior to initiation of study treatment. 12. Treatment with therapeutic oral or IV antimicrobials within 1 week prior to initiation of study treatment. 13. Prior allogeneic stem cell or solid organ transplantation. 14. Treatment with a live, attenuated vaccine or investigational therapy within 28 days prior to initiation of study treatment. 15. Prior treatment with CD137 agonists, anti-CTLA therapeutic antibodies, or an anti-LAG3 agent. 16. Treatment with systemic immunostimulatory agents within 4 weeks (or 5 half-lives) or systemic corticosteroids/immunosuppressive medications within 2 weeks prior to initiation of study treatment. 17. History of severe allergic anaphylactic reactions to chimeric or humanized antibodies or fusion proteins. 18. Known hypersensitivity to Chinese hamster ovary cell products, tobemstomig, pembrolizumab, or any allergy/hypersensitivity to the nab-paclitaxel formulation.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Experimental Arm:* Tobemstomig administered every 3 weeks (Q3W) + nab-paclitaxel administered on a repeating schedule of 3 weeks on, 1 week off. * *Active Comparator Arm:* Pembrolizumab administered every 3 weeks (Q3W) + nab-paclitaxel administered on a repeating schedule of 3 weeks on, 1 week off. **Study Drugs Profile:** Nab-paclitaxel (Paclitaxel; ATC Code: L01CD01. Trade names: Abraxane, Onxol, Taxol, Paccal). **Route of Administration:** Intravenous (IV) infusion for all study drugs. **Duration of Treatment:** Until disease progression, intolerable toxicity, or up to 24 months after the first treatment.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications. **Prohibited:** Treatment with therapeutic oral or IV antimicrobials within 1 week prior to initiation; investigational therapy or live, attenuated vaccines within 28 days prior to initiation of study treatment; prior CD137, anti-CTLA, or anti-LAG3 agents.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures
1	Days -28 to 0	Informed Consent, RECIST evaluation, central testing for PD-L1, ECG, HIV/viral/hepatitis screening
2-5	Q3W	IV infusions of Tobemstomig or Pembrolizumab. Weekly infusions (3 weeks on, 1 week off) of Nab-paclitaxel. Safety Labs, Efficacy Assessments
6	Up to 24 Months	Final tumor imaging, IP Accountability, End of Treatment Review
7	1 Month post-treatment, then Q3 Months	Post-treatment safety check at 1 month. Clinic visits, telephone, or medical record review every 3 months for survival follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * Progression-Free Survival (PFS): Defined as the time from the start of the trial (randomization) until disease progression (cancer worsening) or death from any cause. Measurement Method: Radiographic assessment per RECIST v1.1.

7.2 Secondary Endpoints * Objective Response Rate (ORR): Number of participants whose tumors shrink. *** Duration of Response (DOR).** *** Overall Survival (OS):** How long participants live. *** Incidence and severity of Adverse Events (AEs).**

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for the number and seriousness of any side effects at each clinic visit. **Serious Adverse Events (SAEs):** Reported within standard 24-hour regulatory timelines to the sponsor. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee (DMC) monitors the safety and scientific integrity of the clinical trial and can recommend stopping the trial if it is causing undue harm.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (PFS) analyses; Safety Set for all adverse event tracking. **Sample Size Justification:** $N = 83$ participants powered to estimate the clinical benefit of tobemstomig + nab-paclitaxel relative to the active comparator in the target TNBC population. **Handling of Missing Data:** Standard survival analysis methodologies (e.g., right-censoring) for PFS and OS endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular site visits, source data verification, and remote monitoring by the Sponsor (Roche) to ensure protocol adherence. **Audit Trail:** Centralized laboratory tracking for PD-L1 biomarker expression and electronic Case Report Forms (eCRFs) utilizing validated clinical data management systems.

11. Study Identifiers & Governance

Primary ID: C044194 **Registry Number:** NCT05852691 **Posting ID:** 725224621758113085 **Sponsor/Lead Organization:** Roche **Study Title:** A Study of Tobemstomig + Nab-Paclitaxel Compared With Pembrolizumab + Nab-Paclitaxel in Participants With Previously

Untreated, PD-L1-Positive, Locally-Advanced Unresectable or Metastatic Triple-Negative Breast Cancer **Official Regulatory Title:** A phase II, multicenter, randomized, double-blind study of tobemstomig/R07247669 combined with nab-paclitaxel compared with pembrolizumab combined with nab-paclitaxel in participants with previously untreated, PD-L1 positive, locally-advanced unresectable or metastatic triple-negative breast cancer.

12. Clinical Framework (Oncology Specific)

Primary Condition: Breast Cancer **UMLS Preferred Name:** Triple Negative Breast Neoplasms **Diagnosis Threshold:** Histologically documented TNBC (HER2-low, ER-negative, PgR-negative) with PD-L1 positivity **Medical Methodology:** Targeted Immunotherapy + Chemotherapy **Optimization Target:** Disease control / Progression-Free Survival (PFS)

13. Study Procedures & Methodology

Management Pathway: Intravenous infusion of Tobemstomig or Pembrolizumab (Q3W) combined with Nab-paclitaxel (3 weeks on, 1 week off). **Education Protocol (HCP):** Blinded administration protocols and RECIST v1.1 evaluation training. **Education Protocol (Patient):** IV infusion safety and AE self-reporting instructions. **Quality Audit Mechanism:** Independent DMC safety reviews and central tumor tissue testing for PD-L1 expression.

14. Observation & Follow-Up Schedule

Total Duration: Up to 24 months of treatment, followed by long-term survival tracking (total participation > 2.5 years). **Onsite Visit Cadence:** Frequent regular clinic visits (approximately 10 times every 12 weeks) for IV administration and safety assessments. **Remote Monitoring Frequency:** Follow-up by telephone, medical records, or clinic visits every 3 months after the end of treatment. **Checklist Review Interval:** Routine efficacy evaluation (tumor imaging) every ~6 to 12 weeks.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				